

Benefits, Risks & Governance — NHS ERIC 2024/25

Expected benefits

- B01 — Faster exception identification; proposed measure: time from dataset availability to first identified candidate exceptions.
- B02 — More consistent review; proposed measure: percentage of reviewed records assessed using documented rules.
- B03 — Better contextual investigation; proposed measure: percentage of exception reviews where required context is available at first review.
- B04 — Improved transparency; proposed measure: percentage of exception records with an understandable flag reason.
- B05 — Improved data-quality visibility; proposed measure: number/rate of validated exceptions identified or resolved.
- B06 — Reduced analytical effort; proposed measure: analyst effort per review cycle.

Key risks

- False-positive interpretation — mitigate with potential-exception labels, context and human investigation.
- Unsupported threshold — label £1,000/m² as exploratory and replace with an agreed rule if one exists.
- Data-definition changes — review and version definitions at each refresh.
- Source-data quality — retain source values and investigate unusual observations rather than automatically correcting them.
- Overreliance on dashboard — maintain human-in-the-loop review.
- Access/confidentiality — real deployment requires appropriate organisational access controls and information governance.

Governance

Recommended controls include named data ownership, metric-definition governance, change control, data-quality checks, access control, auditability, UAT sign-off and post-release review.

Change impact

Reviewers move from manually spotting unusual observations toward reviewing a prioritised exception view; analysts use standardised calculations and filters; management interprets flags as investigation prompts rather than performance judgements.